

Prior-Authorization

Determines payer requirements, assembles evidence, evaluates criteria, monitors status — a PA nurse submits.

~39 / wk

prior auths per physician (~13 hours); 40% have staff working only on PA

94%

of physicians say PA delays necessary care; 78% report treatment abandonment

Jan 2027

CMS-0057-F deadline for payer FHIR Prior-Authorization APIs

How we solve it

check_requirement → gather_evidence → evaluate_criteria → assemble_packet → compliance_check → [PA Nurse gate] → finalize

Outcomes

- Lower authorization cycle time
- Fewer care delays / abandonment
- CMS-0057-F readiness
- Consistent, cited clinical rationales

The bright line

The agent submits a PA request behind a human gate — but the coverage determination is the payer's, never the agent's.